

1. Administrative & Study Identification

Official Title: A Phase Ib Study Evaluating the Safety, Efficacy, Pharmacokinetics, and Pharmacodynamics of Glofitamab in Combination With Gemcitabine Plus Oxaliplatin in U.S. Patients With Relapsed or Refractory Diffuse Large B-Cell Lymphoma **Public Study Title:** A Study Evaluating the Safety and Efficacy of Glofitamab + Gemcitabine + Oxaliplatin in U.S. Patients With Relapsed or Refractory Diffuse Large B-Cell Lymphoma **Short Title/ Acronym:** GLOFIT Study (Glofitamab + GemOx in R/R DLBCL) **Protocol Number:** G044900 **NCT Number:** NCT06624085 **Trial Status:** Recruiting **Sponsor/Company Name:** Hoffmann-La Roche (Roche) **Investigational Product:** Glofitamab, in combination with Gemcitabine (Trade Names: Gemzar, Infugem), Oxaliplatin, Obinutuzumab, and Tocilizumab **Phase of Development:** Phase 1 (Phase 1b) **Disease Area:** Lymphoma **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and the complete response (CR) rate of glofitamab in combination with gemcitabine plus oxaliplatin in participants in the United States with relapsed or refractory (R/R) diffuse large B-cell lymphoma (DLBCL). **Secondary Objectives:** To evaluate the objective response rate (ORR), duration of response (DOR), duration of CR, pharmacokinetics (PK), and pharmacodynamics (PD) of the investigational regimen.

2.2 Background & Rationale To address the significant unmet medical need for effective therapies in U.S. patients—including under-represented racial and ethnic populations—with R/R DLBCL who have progressed on or are ineligible for standard treatments. Combining glofitamab (a CD20xCD3 bispecific antibody) with standard gemcitabine and oxaliplatin chemotherapy may generate potent, synergistic antitumor immune responses.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Single Group Assignment) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: 50 participants **Number of Sites:** Approximately 15 locations across the United

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histologically confirmed DLBCL, not otherwise specified (NOS). 2. Relapsed (disease that has recurred following a response lasting $\geq$ 6 months after completion of the last line of therapy) or refractory (disease that did not respond or progressed $<$ 6 months after the last line of therapy) disease with at least one prior line of systemic therapy. 3. Participants who have failed only one prior line of therapy must not be a candidate for high-dose chemotherapy followed by autologous stem cell transplant (ASCT). 4. At least one bi-dimensionally measurable ($>$ 1.5 cm) nodal lesion, or one bi-dimensionally measurable ($>$ 1 cm) extranodal lesion, as measured on a CT scan. 5. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0, 1, or 2.

4.2 Exclusion Criteria 1. Primary or secondary CNS lymphoma at the time of recruitment, or current/history of CNS disease (such as stroke, epilepsy, CNS vasculitis, or neurodegenerative disease). Prior definitively treated CNS involvement in complete remission is permissible. 2. Prior treatment with glofitamab, other bispecific antibodies targeting both CD20 and CD3, gemcitabine, or oxaliplatin. 3. Prior enrollment in Study G041944 (STARGLO; NCT04408638). 4. Participant has failed only one prior line of therapy and is a candidate for stem cell transplantation. 5. History of transformation of indolent disease to DLBCL, high-grade B-cell lymphoma with MYC and BCL2 and/or BCL6 rearrangements, or primary mediastinal B-cell lymphoma. 6. History of severe allergic or anaphylactic reactions to humanized or murine monoclonal antibodies (or recombinant antibody-related fusion proteins) or known sensitivity or allergy to murine products. 7. Peripheral neuropathy or paresthesia assessed to be Grade $\geq$ 2 according to NCI CTCAE v5.0 at enrollment. 8. Known active bacterial, viral, fungal (excluding nail beds), mycobacterial, parasitic, or other infections, including documented SARS-CoV-2 infection within 6 months, positive SARS-CoV-2 test within 7 days, positive hepatitis B (HBV) / hepatitis C (HCV) tests, suspected or latent tuberculosis, known or suspected chronic active Epstein-Barr viral infection, hemophagocytic lymphohistiocytosis (HLH), or progressive multifocal leukoencephalopathy. 9. Prior solid organ transplantation or prior allogeneic stem cell transplant. 10. History of other primary malignancy (with protocol exceptions), significant or extensive cardiovascular disease, significant pulmonary disease, clinically significant cirrhotic liver disease, active autoimmune disease requiring treatment, or recent major surgery (within 4 weeks). 11. Ongoing systemic corticosteroid use putting the patient at increased risk of steroid-related iatrogenic adrenal insufficiency.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Participants receive a single IV dose of obinutuzumab pretreatment 7 days prior to the first dose of glofitamab. This is followed by up to 8 cycles of glofitamab + gemcitabine (Gemzar, Infugem) + oxaliplatin. Finally, participants will receive up to 4 cycles of glofitamab monotherapy (for a total of up to 12 cycles of glofitamab). **Route of Administration:** Intravenous (IV) infusions. **Duration of Treatment:** Up to 12 cycles (Cycle length = 21 days), monitored for up to 3 years.

5.2 Concomitant & Prohibited Therapy Permitted: Intravenous Tocilizumab as needed to manage Cytokine Release Syndrome (CRS). **Prohibited:** Treatment with radiotherapy, chemotherapy, immunotherapy, immunosuppressive therapy, or any investigational agent for cancer within 2 weeks prior to first study treatment. Use of systemic immunosuppressive medications, or treatment with monoclonal antibodies for treating cancer, within 4 weeks prior to the first dose is strictly prohibited.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to -8	Informed Consent, Medical History, Imaging/Biopsy Confirmation, Safety Labs
Pre-treatment	Day -7	IV Obinutuzumab administration
Baseline/C1	Day 1	First Dose of Glofitamab + GemOx, PK/PD sampling, CRS monitoring
Interim	Cycles 2-12	Dosing every 21 days, Safety Labs, Response Evaluation (CT/PET)
End of Study	Up to 3 Years	Final imaging, Exit Interview, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints Definition 1: Incidence of Adverse Events (AEs), focusing on safety and the optimization of the Cytokine Release Syndrome (CRS) profile. **Definition 2:** Complete Response (CR) rate. **Measurement Method:** Clinical safety evaluation per NCI CTCAE and Independent Review Facility response assessment using the Lugano Response Criteria over a 3-year timeframe.

7.2 Secondary & Exploratory Endpoints 1. Objective Response Rate (ORR), Duration of Response (DOR), and Duration of CR evaluated up to 3 years. 2. Pharmacokinetics (PK) and Pharmacodynamics (PD) of the combination regimen.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard intensive oncology monitoring, with a specific focus on Cytokine Release Syndrome (CRS) and neurotoxicity. **Serious Adverse Events (SAEs):** Routine 24-hour expedited reporting to the sponsor. **Data Monitoring Committee (DMC):** Internal safety review committee established by Hoffmann-La Roche to evaluate dose safety profiles.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy (CR rate); Safety-Evaluable Population for incidence of AEs. **Sample Size Justification:** Target $N=50$ is sufficient to describe the safety profile, determine maximum tolerated doses, and evaluate preliminary CR/ORR efficacy signals in a Phase Ib setting. **Handling of Missing Data:** Standard survival and time-to-event statistical censoring applied at the date of the last evaluable tumor assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Standard risk-based oncology monitoring, including onsite and remote data verification. **Audit Trail:** Utilization of secure, FDA 21 CFR Part 11-compliant eCRF systems with rigorous query generation and locking protocols.

11. Study Identifiers & Governance

Primary ID: G044900 **Posting ID:** 362213820902993546 **Registry Number:** NCT06624085 **Sponsor/Lead Organization:** Hoffmann-La Roche (Roche) **Study Title:** GLOFIT Study **Official Regulatory Title:** A Phase Ib Study Evaluating the Safety, Efficacy, Pharmacokinetics, and Pharmacodynamics of Glofitamab in Combination With Gemcitabine Plus Oxaliplatin in U.S. Patients With Relapsed or Refractory Diffuse Large B-Cell Lymphoma

12. Clinical Framework (DLBCL Specific)

Primary Condition: Relapsed or Refractory Diffuse Large B-Cell Lymphoma (R/R DLBCL) **Diagnosis Threshold:** Histologically confirmed disease failure after ≥ 1 line of therapy **Medical Methodology:** Bispecific Antibody (CD20xCD3) combined with DNA-inhibiting systemic chemotherapy **Optimization Target:** CR rate enhancement and safe CRS mitigation

13. Study Procedures & Methodology

Management Pathway: Monoclonal antibody pre-treatment \$ \rightarrow \$ combination chemoimmunotherapy \$ \rightarrow \$ monotherapy maintenance **Education Protocol (HCP):** Site initiation and rigorous training on identifying and treating CRS (utilizing Tocilizumab parameters). **Education Protocol (Patient):** Strict guidance on monitoring for fevers, neurologic changes, and infection risks. **Quality Audit Mechanism:** Continuous central safety review and AE profile auditing.

14. Observation & Follow-Up Schedule

Total Duration: Up to 3 years **Onsite Visit Cadence:** Pre-treatment, Baseline (Day 1), and Day 1 of every 21-day cycle, plus specified scan intervals. **Remote Monitoring Frequency:** Routine surveillance post-treatment. **Checklist Review Interval:** Following every imaging milestone for disease progression.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead	
Statistician	[Name]	_____	[DD-MMM-YYYY]				